RESULT ACCOUNTABILITY LEDGER

The result nobody owns.

The lab is done when it delivers.
We give every report an owner, a clock and a receipt.

A verifiable administrative receipt for Indian hospitals and clinics. No clinical interpretation.

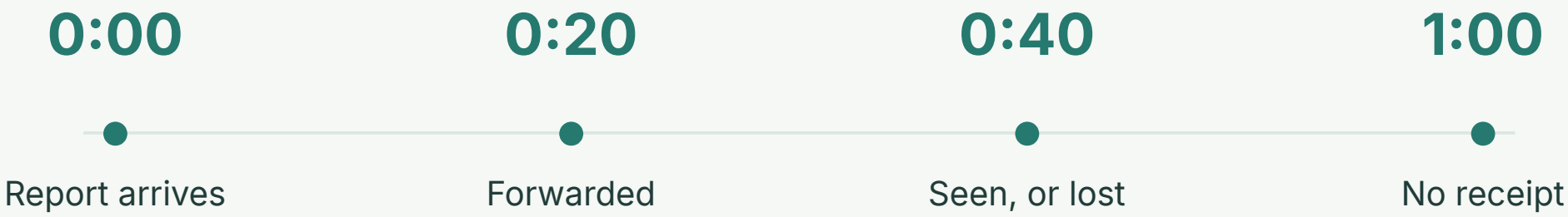


No result without an owner, a clock and a receipt.

PROBLEM

The missing receipt

A report arrives. The lab is finished. The clinic cannot prove a named clinician saw it or that the patient was told.



The failure is not a reminder. It is an unowned result with no clock and no receipt.

1,187 critical values in an Indian tertiary hospital. 64.61% notified 30 to 120 minutes after the sample arrived. PMC4310845.

SOLUTION

The receipt

Four states. One named owner. One auditable proof.

01

Ingest

Every report

WhatsApp, PDF, paper or portal. The lab flag is copied, never calculated.

02

Match

Right patient

Namesake, duplicate or wrong identifier stops for a human.

03

Own

Named clock

A clinician, a due time and an escalation path.

04

Prove

Receipt

Acknowledge, inform the patient, read back critical results, close.

A result without a disposition is not closed. A code verifies the chain without exposing patient data.

THE MOAT

The receipt network

An EMR stores a result. A receipt proves the loop closed. A cross-lab network makes it portable.

THE RECEIPT	A hash-chained event for every owner, clock and acknowledgement.
THE PROOF	A public code verifies the chain without patient data.
THE NETWORK	One receipt standard that labs and hospitals can share.

A record stores. A receipt proves. A network makes it the default.

THE MARKET

The quality buyer

The buyer is the quality head, not the doctor alone. The register is already required.

BEACHHEAD

50-200 beds

Private hospital or nursing home with NABH aspiration.

GOVERNMENT

District

District hospital quality team and a lab partner.

Median acknowledge

25% faster

GATE

Critical read-back

100%

GATE

Unowned results

<10%

GATE

Paid continuation

1 of 3

GATE

Targets are gates, not results. Baseline audit first.

PRODUCT

The loop is built.

The live loop from inbound report to a verified receipt, today.

01
Ingest
Server receipt API

➡

02
Match
Patient match + hold

➡

03
Own
Named owner + clock

➡

04
Acknowledge
Read-back enforced

➡

05
Prove
Hash chain + survey pack

Working today: result ledger, receipt drawer, proof panel, public verifier and CSV survey pack.

The demo has twelve synthetic receipts. One button adds a live inbound sample.

No automatic patient message. No diagnosis. No ABDM claim.

GO TO MARKET

Land and expand

One hospital quality team. One lab channel. One district.

WEDGE

50 to 200 bed hospital with NABH aspiration or a district hospital.

BUYER

Quality head, medical superintendent or lab director.

PRICE

₹5,000 to ₹20,000 per facility / month. Hypothesis.

CHANNEL

Labs, district quality programmes, Tier 2/3.

Pilot price fixed for eight weeks. A paid continuation is the gate, not a survey answer.

THE ASK

Three sites. One proof.

A baseline audit, not a demo.

Clinical lead	Pilot sites and clinical sign-off.
Engineering lead	Receipt chain, API and instrumentation.
Design and product	The live receipt and the survey pack.

THE ASK

Three sites.

100 reports each.

Eight weeks.

The result nobody owns, until someone signs.